



ADHD.ME

findable.

nearby.

continuous.

the wait was never the care.



A person decides to ask for help once. What happens next is six to twelve months of waiting, a bill they pay themselves, and — often — no clear idea who to ask in the first place.

the cost is paid by the person waiting.



6–12 mo

typical wait for an adult ADHD assessment appointment



\$1k–\$2k

common out-of-pocket cost of a private adult assessment



1m+

Australians living with ADHD (2023 Senate inquiry)



\$20bn+

estimated annual cost of ADHD to Australia (2023 Senate inquiry)

Wait and cost figures are indicative, pending source confirmation. Anchors: the AADPA Australian evidence-based clinical practice guideline for ADHD (2022) and the 2023 Senate inquiry into ADHD assessment and support services.

the rule just changed.

DEC 2025

Queensland becomes the first state to let GPs diagnose adult ADHD and initiate treatment — with no mandatory additional training gate.

MAR 2026

NSW opens Stage 2 GP training toward independent diagnosis. South Australia, Western Australia and the ACT follow.

2023

The Senate inquiry's chief recommendation: a National ADHD Framework built on shared and collaborative models of care that widen the GP's role.

the permission changed. the pathway didn't.

A GP is now allowed to carry this. There is still no way for a person to find out which GP near them actually does — and no way for a practice to show that it can.



“So who, near me, can actually do this?”

The question the whole system currently answers with a waiting list.

finding someone was the hard part.

MISSED AT SCHOOL

Went through school without the question ever being asked. Came to it as an adult, unprompted.

WANTED ONE GP

The stated priority was getting connected to a consistent GP who would not be judgemental.

HARD TO RAISE AT ALL

Reported that GPs are currently difficult to discuss mental health with in a non-judgemental way.

COULD NOT FIND WHO DOES IT

Reported extreme difficulty finding which GP does ADHD prescribing, without already knowing someone.

THE GOAL IS FUNCTION

The motivation was to stop underachieving and use their potential — not to collect a label.

First patient interview, 16 August 2026. One participant — a signal to test, not evidence of demand. De-identified findings only, recorded with the participant’s knowledge. The interview programme continues.

one route, end to end.

A patient-facing finder and a practice-facing console on one engine.
Scheduling language throughout — availability, never advice.



01

Say what you need

In your words. Not a quiz, and not a score.



02

See who is near you

GPs who have done the training — by suburb, care area and language.



03

Book the first appointment

Assessment, baseline checks and follow-up with one clinician.

built, and running.

The finder carries a person from “what do I need” to a booked first appointment inside one calm shell. No clinical language on the surface — the compliance linter fails the build if it appears.

Q Search-first and plain-language. Matching is keyed to clinician attributes — never to a patient's symptoms.

ADHD assessment
*that takes
you seriously.*

Describe the GP you're looking for...



[Try a demo scenario >](#)

supply is the other half of the problem.

Practices get demand matching inside their own rules, a record of which GP can do what, and a measurement layer that separates additional visits from the ones that would have happened anyway.

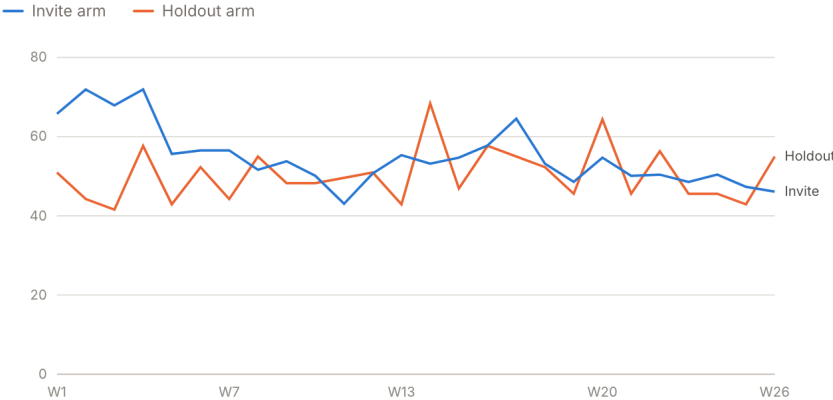
↗ A randomised holdout arm runs by default, so the lift is measured rather than asserted. Every message, booking and opt-out sits on an immutable audit log.

Incrementality

26 simulated weeks · 4,000 synthetic patients

INCREMENTAL ATTENDED / 1,000 120.6 North star — invite- arm rate above holdout	INCREMENTAL ATTENDED 392 vs naive generated count 582 (contrast only)	INVITE ARM 1430 attended / 1,000 · 3,254 patients	HOLDOUT ARM 1310 attended / 1,000 · 746 patients
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Weekly attended per 1,000 — by arm



Attended appointments per 1,000 arm patients per week. Both arms share the same organic process; the gap is the generated lift.

Weekly table

opt-outs 1.2% of 2,952 sent

Synthetic practice engine — 26 simulated weeks, 4,000 synthetic patients. No real patient data exists in this build.

flat fees. never a cut of care.

No percentage of clinical revenue and no per-referral payment, ever. That is a compliance posture — fee-splitting and inducement rules — not a pricing preference.

Solo / small	1–3 FTE GPs	\$4,800
Standard	4–7 FTE GPs	\$9,600
Group	8–12 FTE GPs	\$14,400 – 18,000
Multisite	3 or more sites	\$25,000 – 100,000

AUD, annual, ex GST. 12-week pilot at \$2,400, credited in full against year one on conversion. Buyer breakeven sits at roughly four incremental attended visits per week, practice-wide.

the constraints are the product.



Compliance is code

Copy linters block clinical claims, urgency, benefit claims and testimonials at build time. A non-compliant sentence fails the build — it does not wait for a reviewer to catch it.



Measured, not claimed

A randomised holdout arm runs by default. We report incremental attended appointments against that control — never the raw count of bookings we happened to touch.



The capability graph

Which GP can do what, in which state, under which rule — kept current as each jurisdiction moves. That record is the thing a competitor cannot clone from the outside.

built and verified. pre-pilot, and saying so.

BUILT AND VERIFIED

- ✓ First patient interview complete — discovery programme under way
- ✓ Patient finder and practice console running end to end on a synthetic engine
- ✓ Verify gate on every commit — typecheck, tests, build, dependency audit
- ✓ WCAG 2.1 AA sweep, zero violations, enforced across every public route
- ✓ Pilot playbook, pilot agreement template and pricing model drafted

NOT YET, AND WE WILL SAY SO

No real patient data. No live messages. No pilot practice yet.

Each sits behind a named founder gate (G1–G4) that the build stops at rather than crosses.

The next gate to open is a design-partner practice — which is exactly what we are asking Bond for.

three founders. one of them yours.



Vikram Ganeshalingam

CO-FOUNDER

Final-year MD candidate, Bond University

"What a person meets when they first look for help."



Dr Anubhav Saxena

CO-FOUNDER · MBBS, FRACGP

Practising GP · University of Sydney

"A documented baseline before anything starts, then follow-up on a schedule."



Stefan Thottunkal

CO-FOUNDER

NOURISH, Stanford Medicine · Health Systems Innovation Lab, Harvard T.H. Chan

"Physician-in-training and health-systems researcher."

what we're asking Transformer for.

- 📁 A Launchpad grant toward the pilot quarter — non-equity.
- 🛡️ Two mentors: clinical governance, and an Ahpra advertising review of the name.
- 🏢 Introductions to three Gold Coast general practices as design partners.
- 🧩 Transformer Hub space and coaching through the pilot.

Queensland is the first state where a GP can carry this pathway. Bond is in Queensland.

thanks

Vikram Ganeshalingam

Dr Anubhav Saxena

Stefan Thottunkal

add contact email before sending